STEADY PLATFORM

Demo-First Execution Handoff

Engineering direction for investor review, clinician-supervised synthetic testing, and security audit preparation

Repository	Directive date
tprawlings-lang/Emdr	27 August 2026

Primary instruction
Build as much of the intended platform as can be demonstrated safely with fabricated data. Reviewer approvals do not block T0 investor demonstrations or T1 synthetic clinical and security review. Approval is required before any real-person, public, care-delivery, payer, employee-health, or production use.

Mandatory first action for every coding agent: confirm what the repository already contains and what has already been completed before starting this handoff. Do not rebuild working capabilities. Do not trust this dated snapshot over the live code.

Status: Execution-ready handoff for staged implementation
Classification: Internal planning and demonstration preparation
Data rule: Fabricated and resettable data only

1. Executive directive

The goal is to move Steady from a strong prototype toward a reviewable platform that can be understood by three audiences at the same time: investors, clinicians, and security reviewers. The work should expose the breadth of the intended system without implying that the system is approved for real care or public use.

This handoff changes reviewer questions from build blockers into configurable demonstration assumptions. The team can build and show alternate policy modes using fabricated cases. Clinical, legal, privacy, and security reviewers later decide which modes may be activated in T2 or T3.

Operating rules

- **Proceed:** investor flows, clinician console, payer views, audit views, safety behavior, tenancy, event history, AI summary review, escalation, re-entry, and BLS Part 6 simulation may be built for T0/T1.
- **Label:** every surface must display **DEMO - FABRICATED DATA - NOT CLINICAL CARE**.
- **Separate:** demo-complete, pilot-approved, and production-ready are different states. Never use one as evidence of another.
- **Preserve:** safety checks, crisis resources, stop behavior, grounding closure, kill switch, audit events, and tenant boundaries remain active in demonstrations.
- **Defer activation:** no real participant data, public access, care operations, payer data exchange, or employee-health use until the required review gates pass.

Scope priorities

Priority	Outcome	Reason
1	A coherent end-to-end synthetic demonstration	Lets all audiences see one platform rather than disconnected features.
2	A clinician sandbox with explicit human review	Shows workflow, accountability, and limits without providing care.
3	A security audit environment and evidence packet	Lets an outside security group inspect controls, boundaries, logs, and known gaps.
4	A credible VC data room	Connects vision, delivered code, risk, milestones, and capital needs.

2. Scope, exclusions, and definitions

Tier	Permitted purpose	Data	Who may access	Approval posture
T0 - Investor demo	Guided product demonstration and recorded walkthrough	Fabricated personas and scripted events only	Named internal presenters and invited viewers	Reviewer input is welcomed but does not block build work
T1 - Synthetic review	Clinician workflow testing and security inspection	Fabricated, seeded, resettable datasets only	Named clinicians, security reviewers, engineers, and internal operators	Findings are tracked; no real care or real-person use
T2 - Supervised pilot	Limited real-participant study or clinical pilot	Only approved minimum data	Approved care/research team and participants	Clinical, privacy, legal, security, operational, consent, and protocol gates apply
T3 - Production	Public, employer, payer, or provider deployment	Approved production data	Role-based production users	Full organizational, contractual, regulatory, security, and operational readiness required

Current handoff authorization

- This handoff authorizes planning and implementation for T0 and T1 only.
- It does not authorize clinical care, human-subject research, public launch, payer processing, employer use, claims, diagnosis, treatment, or production handling of protected health information.
- T0/T1 synthetic personas may exercise edge cases outside the eventual pilot eligibility rules when this is needed to prove routing, permissions, safety responses, or reviewer visibility.
- Any screen that resembles a live service must carry the persistent demo banner and a fabricated persona indicator.

Three finish lines

State	Meaning	Evidence required
Demo-complete	The synthetic end-to-end story works, resets reliably, and is honestly labeled.	Scripted journeys, deterministic seed data, build/test record, screenshots, known-gap register.
Pilot-approved	A named reviewer group has accepted the limited T2 design and operating plan.	Ratified review packet, protocol, consent, training, security signoff, incident plan, vendor decisions.
Production-ready	The platform and operating organization can support the intended live market use.	Production controls, contracts, monitoring, support, recovery, audits, privacy and regulatory basis.

A finished demo is evidence of product execution. It is not evidence of clinical effectiveness, legal compliance, security certification, or production readiness.

3. Mandatory codebase preflight

No implementation begins until the coding agent completes and records this preflight against the live repository.

Required inspection

- Record repository, active branch, exact HEAD commit, working-tree status, active pull requests, recent commits, and branch relationships.
- Read the root README, the ADR index, ADRs 0007 and 0009 through 0013, migrations, package scripts, CI workflows, environment samples, and the named clinical and security handoff files.
- Inspect code paths for event append, direct current-state writes, projection rebuild, tenant context, repository access, Postgres RLS, safety chain, audit logging, encryption, AI summaries, demo seeding, and reset behavior.
- Run the available build, unit, safety, end-to-end, RLS attack, migration, dependency, and secret-scanning checks. Record exact commands and outcomes.
- Search for TODO, FIXME, placeholder, mock, bypass, disabled, dormant, and demo-only markers that affect the handoff.
- Confirm whether reported work is merged, only present on a side branch, incomplete, or superseded.

Completion matrix required before coding

Requirement	Status	Evidence	Gap / next action	Owner
Example: projection rebuild	Complete / Partial / Missing / Superseded	Files, tests, command output, commit	Only remaining work	Name
Example: Postgres RLS	Complete / Partial / Missing / Superseded	Schema, policy, attack tests	Dormant until cutover?	Name
Every handoff item	One status only	Direct repository evidence	No duplicated work	Name

First commit or pull-request summary must begin with: branch and HEAD inspected; tests run; completion matrix location; items reused; items changed; new gaps discovered; assumptions that remain provisional.

Conflict rule

- When this handoff and the live code disagree, the agent investigates. It does not silently choose one.
- If the code proves the handoff is stale, update the handoff or add a dated correction in the same change.
- Preserve user or parallel-agent changes. Do not overwrite a dirty worktree or move branch references without confirming the active state.

4. Prior status snapshot to verify

The items below are a planning snapshot from 27 August 2026. They help the next agent know where to inspect. They are not acceptance evidence until the preflight reproduces them on the working branch.

Area	Reported state	Required confirmation
ADR 0010 event spine	Steps 1-4 reported complete: event table, dual-write, genesis backfill, projection rebuild verified byte-identical.	Locate migrations, append code, backfill, rebuild command, comparison tests, rollback path.
ADR 0011 tenancy	Steps 1-6 reported complete: person/account split, TenantContext repository layer, Postgres RLS, 12 attack cases.	Confirm all access paths use tenant context; identify SQLite paths where RLS remains dormant.
Tests	An earlier report cited 327 unit/safety tests and 17/17 end-to-end. A later branch report cited 339 automated tests.	Run the current branch and record the actual count and failures.
Clinical workflow	A draft Handoff B3 workflow was added with demo-first posture and configurable clinical policy modes.	Confirm file exists on the working branch and matches the latest approved wording.
Security package	A later branch added threat, trust-boundary, HIPAA risk, vendor/BAA, identity, logging, incident, and security handoff documents.	Confirm files, owners, current-vs-target labels, evidence links, and unresolved findings.
Important data-flow finding	The companion full stored transcript was reported as decrypted and sent to the model, not merely represented as coded memory.	Trace the live code and correct every architecture, egress, consent, and vendor claim that differs.

Document locations to inspect

- **Clinical:** docs/clinical/steady-clinical-workflow.md and the new clinical-pilot review packet.
- **Security:** docs/security/ and its security handoff README.
- **Architecture:** ADR index, ADR 0007, ADR 0010, ADR 0011, and related schema/migration files.
- **Program direction:** root README, current-vs-target matrix, engineering handoff documents, and platform roadmap.

Do not quote a test count, security control, clinical review, or branch state to investors or reviewers unless it can be tied to the exact demonstration commit.

5. Demo environments and data controls

Control	Investor demo	Clinical sandbox	Security audit environment
Purpose	Narrated product story	Workflow and human-review exercises	Control and attack inspection
Dataset	Small scripted personas	Rich synthetic caseload and edge cases	Synthetic tenant and adversarial fixtures
Reset	One-click story reset	Scenario-level and full reset	Rebuild from clean baseline
Access	Presenter and invited viewer	Named clinician/reviewer accounts	Named security reviewers and operators
Observability	Curated status and outcome view	Timeline, alert, decision, citation, override	Raw audit, control, policy, and failure evidence
External services	Pinned or clearly marked simulated calls	Approved sandbox services only	Isolated test endpoints; no live PHI
Banner	Persistent demo banner	Persistent synthetic sandbox banner	Persistent audit sandbox banner

Fabricated-data rules

- Synthetic people must not be based on a real user, patient, employee, clinician, or identifiable combination of facts.
- Use reserved demo email domains, phone patterns, dates, identifiers, and payer/provider names.
- Never paste real conversation content, health history, screenshots, logs, tokens, keys, or production exports into the demo.
- Seed data must be deterministic, versioned, and reviewable. Resets must remove prior synthetic activity and recreate the expected baseline.
- Telemetry, error reporting, AI vendors, and analytics must use demo projects or be disabled when their data path is not approved.

Non-removable demonstration safety floor

- Pre-model crisis and acute-risk check
- Crisis resources and emergency direction
- Output guard and unsafe-content handling
- Stop control and grounding closure
- Kill switch and feature disable controls
- Immutable or append-only audit evidence
- Tenant boundaries and least-privilege role behavior

6. Configurable clinical policy modes

The team should implement policy decisions as versioned configuration for T0/T1 so reviewers can compare alternatives. Defaults below are demonstration assumptions, not clinical approval.

Policy	Supported modes	T0/T1 default	T2 decision needed
Companion conversation visibility	never / escalation excerpt / member-shared / always	Escalation excerpt only; minimum content; reason and audit	Consent, role, minimum necessary, member visibility, retention, model egress
Caseload model	owned / pooled / hybrid	Hybrid: primary owner plus tenant coverage pool	Accountability, staffing, licensing, handoff, coverage
Out-of-hours coverage	none / business hours / extended / 24-hour	Business hours; no promise of 24/7 clinical monitoring	Operating schedule, member language, escalation destination
Immediate alert consequence	notify / pause processing / lock workflow / emergency path	Pause BLS or processing; retain check-ins, grounding, and crisis actions	Named responder, deadline, severity bands, adverse-event rules
Re-entry	automatic / timed / clinician decision	Clinician decision plus fresh readiness, safety plan, consent, and cooldown	Protocol and BLS-specific clearance
Autonomous engine	off / shadow / recommend / active	Shadow for new engine; deterministic safety chain remains active	Promotion evidence by capability and rollback authority

Clinical workflow principles

- Caseload is ordered by clinical need, not contract tier.
- Immediate-band alerts have a named owner, deadline, and visible status. They do not auto-resolve.
- AI summaries cite their source events. A claim that cannot be cited is not displayed.
- Approve, correct, and override remain separate actions with separate audit events.
- The member-facing coverage statement must match the configured operating schedule.
- Adverse-event definitions are separate from routine safety and operational events.

The three highest-impact reviewer decisions are companion-content visibility, caseload ownership, and out-of-hours coverage. They do not block T0/T1 construction. They do block final T2 permissions, accountability, consent, and operating language.

7. Shared platform foundation

Capability	T0/T1 build target	Acceptance evidence
Identity and tenancy	Organization, person, account, role, and TenantContext enforced through repositories; Postgres RLS ready and active in audit environment.	Cross-tenant attack suite; role matrix; denial logs; no unscoped repository path.
Event spine	Event append and projections support demo workflows, replay, timeline, and audit.	Deterministic replay; projection comparison; idempotency; migration and rollback rehearsal.
Safety chain	Deterministic checks wrap model use and can be exercised by synthetic cases.	Named tests for stop, crisis, grounding, unsafe output, and kill switch.
AI summaries	Cited summary with approve, correct, override, and no-uncited-claim behavior.	Source linkage; action audit; model/version/prompt record; failure behavior.
Audit	Actor, tenant, subject, purpose, action, prior/new state, time, request, and policy version recorded.	Reviewer can trace a full synthetic journey and export evidence.
Configuration	Policy modes are versioned, environment-scoped, and visible in the UI.	Config change audit; deterministic seeded defaults; no hidden production toggle.
Demo operations	Seed, reset, health check, backup, restore, and guided scenario scripts.	Clean reset; known baseline hashes; recovery rehearsal; runbook.

Architecture rule for reviewability

Every important screen state must be explainable from a small set of versioned facts: who acted, which tenant and person were affected, which event occurred, which policy was active, which code and model version ran, what human action followed, and what the system displayed.

Build order within the foundation

- Finish preflight and completion matrix.
- Choose the exact T0/T1 deployment and secret-store pattern; document it without placing secrets in the repository.
- Activate Postgres, tenancy, and RLS in a reproducible non-production environment.
- Make event, projection, and audit writes atomic or prove compensation and replay behavior.
- Add deterministic seed/reset operations and scenario fixtures.
- Connect the same foundation to investor, clinical, payer, BLS, and security views.

8. Investor demonstration package

Narrated product journey

- Create a synthetic organization and member with explicit demo consent state.
- Show companion experience, safety behavior, progress events, and controlled model use.
- Show the clinician caseload, cited summary, alert ownership, approval/correction/override, and re-entry decision.
- Show a payer or enterprise population view using fabricated aggregate data and no individual clinical claim.
- Show tenant isolation, event history, reviewer audit trail, and kill-switch response.
- Show BLS Part 6 as a labeled simulation with its own review and activation gates.

VC data-room deliverables

Artifact	Minimum content
Current-vs-target capability matrix	Delivered, partial, planned, dependent, and excluded items tied to repository evidence.
Architecture and data-flow packet	System context, six governance zones, trust boundaries, event spine, tenancy, model/vendor egress.
Product demo guide	Script, personas, expected states, reset steps, fallback recordings, known limitations.
Clinical pathway packet	T0/T1 workflow, provisional decisions, T2 approval gates, BLS position, reviewer questions.
Security packet	Threats, risks, controls, evidence, vendor/BAA register, incidents, recovery, known gaps.
Roadmap and use of funds	Workstreams, dependencies, staffing, vendors, external reviews, pilot and production gates.
Evidence index	Exact commit, tests, build, scans, screenshots, recordings, review dates, owners.

Investor language must stay exact: "built for demonstration," "synthetic review environment," or "planned control" are acceptable. Do not say "clinically validated," "HIPAA compliant," "secure," "approved," or "production-ready" without the corresponding evidence and authority.

9. Clinician synthetic testing package

Surface or workflow	What the reviewer must be able to test
Caseload	Sort by clinical need, filter by tenant/team/status, see primary owner and coverage pool.
Member timeline	See authored events, source, time, policy, human action, and clear separation from AI output.
Alerts	Named owner, severity, deadline, acknowledgement, resolution evidence, no immediate-band auto-resolution.
AI summary	Citations, uncertainty, omitted uncitable claims, approve/correct/override as distinct actions.
Companion access	Compare configured modes, minimum excerpt behavior, access reason, member visibility, and audit.
Coverage and handoff	Operating hours, out-of-hours message, transfer of accountability, overdue behavior.
Pause and re-entry	Deterministic pause, retained safety/check-in access, readiness reassessment, documented clinician decision.
Feedback	Clinical usefulness, safety concern, factual correction, workflow issue, policy question, and adverse-event classification.

Synthetic scenario set

- Routine progress with no alert and a fully cited summary.
- Uncitable AI claim suppressed before display.
- Immediate alert during and outside configured coverage hours.
- Companion excerpt access allowed, denied, and later audited.
- Primary clinician unavailable and coverage pool takes ownership.
- Processing paused, member uses grounding, then re-entry is approved or denied.
- Cross-tenant access attempt and privilege change attempt.
- BLS Part 6 simulation that reaches a stop or escalation condition.

New clinical review packet

Preserve the prior consumer review record **beta-clinrev-2026-07**. It does not cover multi-tenancy, the clinician surface, clinical-record summaries, the payer view, or BLS Part 6. Create a separate proposed packet named **clinical-pilot-2026-09** for the future T2 design.

10. Security audit package

Artifact	Required contents	Status method
System context and trust boundaries	Actors, services, stores, third parties, six governance zones, ingress/egress, admin paths.	Current / Dormant / Planned
PHI and sensitive-data flow	Fields, purpose, source, destination, encryption, retention, deletion, model/analytics egress.	Trace to code and config
Threat model and abuse cases	Tenant escape, privilege misuse, prompt injection, data poisoning, model leakage, export, replay, insider risk.	Owner, severity, treatment
Risk analysis	HIPAA-aligned administrative, physical, and technical risks without claiming certification.	Likelihood, impact, control, residual risk
Identity and privilege matrix	Roles, grants, denials, break-glass, join/leave, service accounts, tenant admin.	Attack tests and logs
Vendor and BAA register	Vendor, data, purpose, region, contract, BAA need/state, subprocessor, deletion, exit plan.	Evidence link or open decision
Logging and monitoring	Security events, audit schema, alert ownership, clocks, retention, tamper resistance, review.	Demonstrated synthetic event
Incident and recovery	Triage, containment, breach assessment, communication, backup, restore, RTO/RPO targets.	Tabletop and restore result

Audit-environment demonstrations

- Twelve or more cross-tenant and privilege attack cases execute against Postgres RLS and application controls.
- Event replay and projection rebuild reproduce the same synthetic state.
- Secret scan, dependency scan, build provenance, environment inventory, and configuration diff are available.
- A reviewer can follow one synthetic alert from creation through access, decision, correction, export, and retention behavior.
- Known gaps are listed with owner, target tier, acceptance test, and mitigation. Planned controls are never shown as active controls.

Security documentation must match actual code paths. The reported companion-transcript model egress is a specific item to trace and disclose accurately.

11. BLS Part 6 remains in scope

BLS Part 6 stays active as a parallel clinical-validation workstream. Removing it from the roadmap is not part of this handoff. T0/T1 may demonstrate the intended workflow with fabricated scenarios. Real participant access remains gated by its own evidence and approvals.

T0/T1 build now	Do not claim or activate yet
Synthetic protocol states and transitions	Clinical effectiveness
Eligibility and exclusion simulation	Final participant eligibility
Pause, stop, escalation, grounding, and re-entry paths	Autonomous real-person operation
Clinician review queue and evidence capture	Approved clinical protocol
Versioned content and policy selection	Final content approval
Outcome and safety-event instrumentation	Validated outcome interpretation
Labeled payer/investor narrative	Coverage, reimbursement, or medical claims

BLS evidence workstream

- Define the intended mechanism, protocol boundaries, exclusions, stop rules, supervision, and re-entry logic.
- Map each protocol claim to literature, expert review, product behavior, and unresolved evidence.
- Create synthetic cases for normal, ambiguous, unsafe, and failure conditions.
- Separate routine safety signals, product incidents, adverse events, and protocol deviations.
- Record the exact content, policy, model, code, and reviewer version for every demonstrated case.
- Prepare the questions that must be ratified before T2, including clinical ownership and monitoring expectations.

All BLS screens and scripts must state that the workflow is simulated, uses fabricated data, and is not approved clinical care.

12. ADR 0010 Step 5 execution plan

Target execution window: 14-18 September 2026. Step 5 removes direct writes to current-state tables so event append becomes the only write path. It is a technical integrity cutover. It is not waiting on the clinical policy decisions in this handoff.

Gate	Pass evidence
1. Current code confirmed	Preflight matrix complete on the exact cutover branch and commit.
2. Postgres active	Target environment uses Postgres; schema and migrations are reproducible.
3. TenantContext enforced	No application repository path operates without explicit tenant scope.
4. RLS active	Policies run under the application role; attack suite denies cross-tenant access.
5. Atomic write path	Event, projection, and audit behavior cannot leave silent partial state.
6. Projection parity	Rebuilt projections match dual-write state across demo and adversarial fixtures.
7. Backfill and idempotency	Genesis/backfill can be repeated safely and produces the expected hashes/counts.
8. Rollback rehearsed	The team can restore the prior write posture and data state within the agreed window.
9. Observability ready	Append failures, lag, rebuild mismatch, and tenant-policy denial are visible and owned.
10. Synthetic soak	Seven consecutive clean synthetic-staging days after all prior gates pass.

Cutover decision rule

Execute Step 5 in the target window only when all ten gates are evidenced on the same release candidate. If any gate fails, keep dual-write active, record the failure and owner, correct it, restart the affected evidence period, and reschedule. Dual-write is a valid temporary resting place but not a permanent architecture.

Do not accelerate Step 5 merely to make the investor demo look complete. The demo can run in dual-write while the event-authoritative cutover earns its own evidence.

13. Implementation sequence and workstreams

Sequence	Workstream	Deliverable	Exit condition
0	Repository truth	Preflight, completion matrix, exact baseline	No material unknown about what already exists
1	Environment	T0/T1 hosting, secrets, Postgres, seed/reset, demo banner	Reproducible isolated baseline
2	Shared controls	Tenant, event, audit, safety, config, observability	Foundation tests pass
3	Clinical workflow	Caseload, timeline, alerts, summary review, pause/re-entry	Synthetic scenario set passes
4	Investor and payer story	Guided journey, aggregates, architecture and evidence views	Narrative works end-to-end
5	Security review	Evidence packet, attack suite, trace, scans, recovery rehearsal	Outside reviewer can begin audit
6	BLS Part 6	Labeled simulation, protocol states, safety and evidence capture	Demonstration and review packet ready
7	Step 5 cutover	Event-authoritative writes	Ten gates plus seven clean synthetic days
8	Review cycle	Findings, fixes, demonstrations, updated known-gap register	T0/T1 release candidate accepted internally

Parallel execution guidance

- Clinical workflow, security documents, demo UX, and BLS evidence work can proceed in parallel after the shared data contracts and configuration model are stable.
- Do not create separate truth models for investor, clinician, payer, and security views. They should read the same synthetic events and projections with role-specific access.
- Every pull request must state which tier it supports, which data class it permits, which policy assumptions it uses, and how it is tested.
- A feature is not done when the screen renders. It is done when seed/reset, authorization, audit, failure, accessibility, and reviewer evidence are included.

Founder decisions needed without stopping the demo build

Name the clinical reviewers, legal/privacy counsel, security review group, T0/T1 hosting provider, secret store, infrastructure budget, and vendor inventory. Until named, the team records an owner as **Founder - TBD**, uses the provisional T0/T1 defaults in this handoff, and continues only with fabricated data.

14. Acceptance criteria for T0/T1 release

Area	Release acceptance
Repository	Exact demo commit identified; completion matrix current; clean build; tests and scans recorded; no unreviewed secret.
Data	Only fabricated data; deterministic seed; full reset; no copied real records; demo indicators visible.
Investor	Guided journey runs without manual database edits; fallback recording exists; claims match evidence.
Clinical	All scenario cases run; AI claims are cited or suppressed; human actions are distinct and audited; no implied monitoring promise.
Security	Tenant attacks, privilege denials, event replay, audit trace, dependency/secret scans, and restore rehearsal are available.
BLS	Simulation is complete enough for review; stop, pause, escalation, grounding, and re-entry paths are testable and labeled.
Operations	Access list, runbook, incident contact, backup/restore, health checks, kill switch, and environment owner are defined.
Documentation	README points to execution, clinical, security, ADR, demo, and data-room documents; current/planned status is explicit.
Review honesty	Known gaps, provisional policies, dormant controls, and T2/T3 gates are visible in the product and packet.

Release evidence bundle

- Demo commit and build identifier
- Test and scan reports with exact commands
- Seed dataset version and reset verification
- Investor demo script and recording
- Clinician scenario results and open questions
- Security evidence index and known-gap register
- BLS simulation results and approval exclusions
- Architecture and sensitive-data flow matched to current code
- Reviewer access list and environment expiration or shutdown plan

Any real-person information found in a T0/T1 environment is a stop condition: isolate the environment, preserve evidence, notify the named owner, assess exposure, remove the data through the approved process, and do not resume until the cause is corrected.

15. Handoff checklist for the coding team

Check	Required record
[ ] Confirm live repository first	Branch, HEAD, dirty state, PRs, recent commits, active documents, migrations.
[ ] Produce completion matrix	Complete / Partial / Missing / Superseded with direct evidence.
[ ] Select T0/T1 environment pattern	Hosting, secret store, database, model sandbox, analytics, access, expiration.
[ ] Enforce fabricated-data boundary	Seed policy, reserved identifiers, banner, reset, telemetry check.
[ ] Preserve safety floor	Crisis, guard, stop, grounding, kill switch, audit, tenancy tests.
[ ] Implement versioned policy modes	Companion access, caseload, coverage, alert, re-entry, autonomous mode.
[ ] Complete three review experiences	Investor demo, clinical sandbox, security audit environment.
[ ] Keep BLS Part 6 active	Labeled simulation plus separate T2 activation gates.
[ ] Prepare Step 5	Ten technical gates and seven clean synthetic-staging days.
[ ] Package evidence	Exact commit, tests, scripts, flows, risks, current-vs-target labels, known gaps.
[ ] Update README and indexes	Clear path to strategy, execution documents, clinical, security, ADR, and demo runbook.
[ ] Stop at T2 boundary	No real-person, public, clinical-care, payer, employee-health, or production use.

Final direction

Build the platform story now. Use fabricated data, explicit labels, versioned assumptions, strong audit evidence, and repeatable demonstrations. Let clinicians and security reviewers inspect a working system. Keep their findings tied to future activation gates. Do not let unanswered policy questions prevent T0/T1 progress, and do not let a polished demo be mistaken for permission to use the system with real people.

End of handoff
Owner: Founder / Steady
Repository: tprawlings-lang/Emdr
Directive date: 27 August 2026
Next formal checkpoint: repository preflight and completion matrix